CQC MEDISOLUTIONS | FREE GUIDE

# Systems down?

## The CQC Business Continuity Checklist

A printable guide for every CQC registered provider: dental, hair restoration and aesthetic clinics, IV therapy, private GP, domiciliary care, supported living, care homes and more.

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BEFORE YOU START

# Why CQC cares about this

When Microsoft 365 went down in November 2026, thousands of businesses lost email, Teams and files in one go. For a care provider, that is not an inconvenience. It is a risk to the people you look after.

CQC expects every registered provider to plan for disruption, so that people keep receiving safe care when things go wrong. It links to **Regulation 12** (safe care and treatment) and **Regulation 17** (good governance), and to the **Safe** and **Well-led** key questions. Inspectors may ask to see your business continuity plan, ask staff what they would do in a real disruption, and look at how you handled incidents that already happened.

**The question applicants and managers dread:** "What happens if your systems go down?" It comes up in registered manager interviews and on inspection. A strong answer is specific to your service, names who does what, and proves you have tested it. Part 6 shows you how to answer it.

## How to use this guide

1. Work through **Part 1** to check your plan has the essentials.
2. Go through every scenario in **Part 2**. Tick what you already have in place. Anything unticked is your action list.
3. Add the page for your sector in **Part 3**. Run more than one type of service? Use every page that applies.
4. Build your offline **grab pack** (Part 4) so your team can work with no systems at all.
5. Test the plan and record it in the **log** (Part 5). A plan that has never been tested is the first thing an inspector will probe.

**Keep it secure.** Paper copies of contact lists, care records, key safe codes and medical histories are personal and sensitive data. Store them locked away, limit who can access them, and shred old versions when you print new ones.

PART 1

# Your plan essentials

Every provider, every sector. If you can tick all of these, you have a real plan rather than a template.

## The plan itself

- [ ] A written business continuity plan that names **your** service, **your** location(s) and **your** people, not a generic template
- [ ] A named person who activates the plan, plus a named deputy for when they are unavailable
- [ ] A risk assessment of what could disrupt your service, rated by likelihood and impact
- [ ] A list of **critical activities** that must never stop (for example medicines, personal care visits, urgent treatment) and those that can safely pause
- [ ] A priority list of the people who use your service who are most at risk if care stops or is delayed
- [ ] Clear links to your related policies: emergency, fire, infection control, data security, lone working, safeguarding

## Contacts and communication

- [ ] Up to date contact list: staff, on-call manager, IT support, suppliers, landlord, utilities, insurer, commissioners or local authority
- [ ] A phone cascade so staff can be reached if email, Teams or your app is down (personal mobiles, WhatsApp group or text, agreed in advance)
- [ ] A way to contact people who use your service and their families if your normal system fails
- [ ] Template messages ready to send: service delay, appointment cancellation, temporary closure

## Regulatory and insurance

- [ ] You know when you must notify CQC, including events that stop, or may stop, you running the service safely and properly
- [ ] You know that a registered manager absence of 28 days or more must be notified to CQC
- [ ] Insurance checked: business interruption and cyber cover, and what your policy needs from you to make a claim

## Storage, training and review

- [ ] The plan is stored in at least two places, including a printed copy that can be reached if the building or systems are unavailable
- [ ] Every member of staff knows where the plan is and what their role is (covered at induction and refreshed)
- [ ] The plan is reviewed at least once a year, after any incident, and after any significant change to your service

PART 2

# Nine disruption scenarios

Tick what is already in place. Anything left blank goes on your action list.

## 1. IT, software or internet outage

- [ ] You know which systems hold critical information: care records, eMAR, rota, booking or practice management software, phones
- [ ] Paper downtime forms are ready: care notes, MAR charts, day list or appointment list, incident forms
- [ ] Essential information is printed or exported on a set schedule (for example weekly) so it is never more than a few days old
- [ ] Staff can see the rota and visit or appointment schedule without the app
- [ ] A backup internet option is available (for example a mobile hotspot)
- [ ] Your IT provider's emergency contact and your software suppliers' status pages are listed
- [ ] A process to write paper records back into the system once it is restored, and someone responsible for doing it

## 2. Cyber attack or data breach

- [ ] Staff are trained to spot phishing emails and know who to tell straight away
- [ ] Data is backed up automatically and you have tested restoring it
- [ ] Multi-factor authentication is switched on for email and key systems
- [ ] Steps to isolate an infected device are written down
- [ ] You know a reportable personal data breach must be reported to the ICO within 72 hours
- [ ] Data Security and Protection Toolkit completed, where it applies to you (for example if you hold NHS data or contracts)

## 3. Power, water, heating or gas failure

- [ ] Emergency numbers listed: 105 for power cuts, plus your water and gas suppliers
- [ ] Torches and battery lighting kept where staff can find them in the dark
- [ ] Medicines fridge plan: temperature monitoring and where stock goes if the fridge fails
- [ ] Critical equipment that needs power is identified, with backup (battery, UPS or generator) where needed
- [ ] Plan for keeping people warm, clean and hydrated, including bottled water and hand hygiene without running water
- [ ] Clear point at which you stop treatment or close, and who makes that call

## 4. Staff shortage (sickness, outbreak, weather, strike)

- [ ] Minimum safe staffing levels are written down
- [ ] Bank and agency contacts are listed, with agency checks you can rely on
- [ ] A skills matrix shows who can cover what (medication trained, drivers, clinicians, first aiders)
- [ ] A plan for prioritising visits or appointments when you are short
- [ ] A named deputy for the registered manager

## 5. Loss of premises (fire, flood, structural damage, no access)

- [ ] Evacuation plan and fire risk assessment are current
- [ ] Alternative premises, or an arrangement with a partner service, is identified
- [ ] You know that operating from a new location usually needs a change to your CQC registration, so you would contact CQC early
- [ ] Key equipment, records and medicines can be protected or moved

## 6. Supplier failure (medicines, PPE, consumables, equipment, labs)

- [ ] At least one alternative supplier for each critical item
- [ ] Minimum stock levels set and checked
- [ ] Servicing and repair contacts for critical equipment are listed

## 7. Severe weather and travel disruption

- [ ] You check weather warnings and act early
- [ ] Staff who live close to people or sites are identified for cover
- [ ] Alternative travel options are agreed (lifts, 4x4, public transport, walking routes)

## 8. Infection outbreak

- [ ] Outbreak plan sits alongside your infection prevention and control policy
- [ ] PPE stock levels are monitored with a reorder point
- [ ] Your local UKHSA health protection team contact is listed
- [ ] Plan for isolating people and cohorting staff where relevant

## 9. Loss of a key person

- [ ] No single person holds the only password, key, code or bank access
- [ ] Passwords are held securely in a shared password manager or sealed envelope system
- [ ] More than one person can sign off payroll and pay suppliers
- [ ] Key processes are written down so someone else can follow them

PART 3

# Sector add-ons

Use the page or pages that match your service. These sit on top of Parts 1 and 2, not instead of them.

## Dental practices

- [ ] Patients in pain can still get urgent care during a closure (arrangement with another practice, or clear signposting to NHS 111)
- [ ] Practice management software down: a printed day list and secure access to medical histories for that day's patients
- [ ] Autoclave or washer disinfector failure: stop treatment that needs sterile instruments, switch to single use where safe, engineer contact listed
- [ ] Compressor or suction failure: service engineer contact and the point at which treatment stops
- [ ] X-ray equipment failure: Radiation Protection Adviser and engineer contacts listed
- [ ] Emergency drugs, oxygen and AED checks continue during any disruption

## Clinics: hair restoration, aesthetics, IV therapy, private GP and similar

- [ ] Power failure mid-procedure: a plan to safely complete or stop, with emergency lighting and battery-powered equipment
- [ ] Hair restoration: plan for safe graft handling and storage if power or equipment fails during a procedure
- [ ] Cold chain for medicines and products (for example botulinum toxin, vaccines): fridge monitoring and a backup fridge or arrangement
- [ ] Aftercare line stays staffed if phones or email go down, so patients with complications can reach a clinician
- [ ] Cover arrangements if your prescriber or lead clinician is unavailable
- [ ] Access to test results and pathology if your lab or portal is down

## Domiciliary care and supported living

- [ ] Electronic call monitoring down: staff log visits by phone call or text to the office, recorded on a paper log
- [ ] eMAR down: paper MAR charts ready for every person who receives medication support
- [ ] A secure offline copy of the visit schedule, addresses, access information and key safe codes
- [ ] A priority list of people who cannot be missed: time-critical medicines, insulin, two-carer calls, people who live alone with no family
- [ ] Lone worker check-ins still happen when the app is down
- [ ] Commissioners, local authority and families are told quickly if visits will be late or missed
- [ ] Plan for vehicle breakdown or fuel shortage

## Care homes and residential services

- [ ] Personal emergency evacuation plans (PEEPs) are current for every resident
- [ ] An agreement for temporary accommodation if residents must leave the building
- [ ] Food, water and continence supplies kept for at least 72 hours
- [ ] Lift failure plan for residents on upper floors
- [ ] Oxygen and other critical medical supplies have a backup source
- [ ] Generator or plan for heating, lighting and kitchen if the power fails

## Any other service

- [ ] You have listed the one or two things that would hurt people most if they stopped, and planned for those first
- [ ] Your plan covers how people who use the service will know what is happening and who to contact

PART 4

# The offline grab pack

One folder or box your team can pick up and work from with no computer, no internet and no building access. Keep it locked, keep it current, and keep a second copy off site.

## What goes in it

- [ ] Printed business continuity plan, with the activation steps on the first page
- [ ] Staff and emergency contact list, including the phone cascade
- [ ] Current rota and visit or appointment schedule
- [ ] Priority list of the people most at risk
- [ ] Key medical information and medication details for those people (paper MAR charts where relevant)
- [ ] Blank downtime forms: care notes, MAR, day list, incident forms, visit log
- [ ] Supplier, engineer, landlord and utility contacts
- [ ] Location of utility meters, stopcock, fuse board and gas isolation valve
- [ ] Simple floor plan showing exits, fire equipment and emergency kit
- [ ] IT recovery steps and your IT provider's contact details
- [ ] Template messages for staff, people who use the service, families and commissioners

**Grab pack last updated:** _____ **Checked by:** _____

**Off-site copy held by:** _____ **Next review due:** _____

PART 5

# Test and review log

Test at least once a year. A 30 minute tabletop exercise counts: pick a scenario, talk it through with your team, and write down what you learned. Log real incidents here too. This page is your evidence.

| Date | Scenario | Test or real incident? | What went well | What we will improve | Owner | Done |
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**Ideas for a first test:** “It is 7am and the care app will not load.” “The autoclave fails at 10am with a full day list.” “The power goes off halfway through a procedure.” “Half the team call in sick on a snow day.”

PART 6

# “What happens if your systems go down?”

How to answer in a registered manager interview or on inspection. Inspectors are listening for five things.

1. **What is critical.** Name the systems and the information that matter most. “Our care records, eMAR and rota are on [system].”
2. **What we do straight away.** The first steps, in order. Switch to paper downtime forms, start the phone cascade, check the priority list.
3. **Who is in charge.** The named person who activates the plan, and their deputy.
4. **How people stay safe.** How you make sure the people most at risk still get their care and medicines on time.
5. **How we recover and learn.** Writing records back in, reviewing what happened, updating the plan, and when you last tested it.

> **Example answer (home care):** “Our care plans, eMAR and rota are on [system]. If it goes down, the on-call manager activates our continuity plan. Staff switch to the paper MAR charts and visit logs in our grab pack, which we update every week, and log in and out of visits by calling the office. We go straight to our priority list, so people on time-critical medicines and two-carer calls are covered first. Once the system is back, the office enters the paper records the same day and we review what happened. We last tested this in [month] with a tabletop exercise and changed our cascade as a result.”

Use this as a structure, not a script. Inspectors can tell when an answer has been memorised from someone else’s template. Fill it with your real systems, your real people and your real test.

Disclaimer
This guide is general information to help you plan. It is not legal advice and does not replace CQC’s own guidance, which you should check for your service type. Correct at the time of writing (November 2026).

FROM CQC MEDISOLUTIONS

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